SYNTHETIC RECORD - FOR DEMONSTRATION ONLY
Patient: Tomas Brightwater. DOB: 1957-06-15. MRN: SYN-100603.
Visit date: 2026-08-28. Clinic: Northgate Endocrinology (fictional).
Diagnosis: type 2 diabetes mellitus (E11.65), long-term insulin use (Z79.4).
Insulin glargine 32 units at bedtime daily since 2023.
Insulin lispro 6-8 units with meals three times daily since 2025-02.
Checks fingerstick glucose 4 times daily.
Two overnight episodes of glucose 48 mg/dL and 51 mg/dL in July 2026.
Spouse assisted during one hypoglycemia episode.
HbA1c 8.9% on 2026-08-20.
Today is an in-person visit with the treating endocrinologist to review glycemic control.
Medications: insulin glargine, insulin lispro, metformin 1000 mg BID, losartan 50 mg daily.
Exam: BP 128/76, weight 92 kg. Injection sites without lipohypertrophy.
Demonstrates understanding of insulin dose adjustment.
Diet: consistent carbohydrate meal plan.
Assessment: insulin-requiring type 2 diabetes with level 2 hypoglycemia.
Plan: order a therapeutic continuous glucose monitor (E2103) with sensors (A4239).
CGM prescribed for use according to its FDA-cleared indications, to improve glycemic control.
CGM will guide insulin titration and provide nocturnal hypoglycemia alerts.
Patient completed CGM device training with the diabetes educator today; I judge the training sufficient.
Hypoglycemia treatment plan reviewed; glucagon prescribed.
Follow-up in 6 weeks with CGM data download.
Electronically signed: Avery Lindqvist, MD (fictional).